



PROCEDURE

ECT Preparation and Transfer Process for Older Adult Inpatients

Scope (Staff):	All MHPHDS Older Adult Mental Health Inpatient Staff
Scope (Area):	All MHPHDS Older Adult Mental Health Inpatient Services

Contents

1. Aim.....	1
2. Background.....	1
3. Risk.....	2
4. Process.....	2
4.1 Day Prior to ECT	2
4.2 Night Staff Preparation of Patient.....	3
4.3 Day of ECT Preparation	3
4.4 Post ECT	3
4.4.1 Post ECT (Prior to leaving ECT Suite).....	3
4.5 Post ECT Nurse Role on return to Inpatient facility	4
4.5.3 Post ECT patients should be advised not to:.....	4
4.6 Management of Ongoing ECT Treatment/Ceasing Treatment	4
4.6.1 Ongoing ECT Management	4
4.6.2 Suspension, postponement or ceasing of ECT	5
5. Compliance, monitoring and evaluation	5
6. Appendix 1: Medication during ECT.....	8
7. Appendix 2: Post ECT Inpatient Facility Checklist	11

1. Aim

This procedure provides guidance to ensure a standardised approach to the nursing and medical process for preparing a transferred inpatient for Electroconvulsive Therapy (ECT). This procedure is applicable to all Older Adult Mental Health Inpatient Services.

2. Background

Older Adult Mental Health Inpatient Services are responsible for the preparation and facilitation of inpatient transfers to external facilities to access ECT.

3. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Process

4.1 Day Prior to ECT

On the day prior to ECT the Clinical Nurse Specialist (CNS) alongside the Treating Team will review all ECT documentation and check that it is completed and available including:

- Appointment confirmed in the Shift Coordinator Diary, Wing Diary and Medical Records Diary
- ECT Consent Form (PMR 73)
- ECT Treatment Form (PMR 73)
- ECG Tracing
- CXR
- Bloods (available OL via iSOFT)
- Old Notes and any past ECT treatment charts and outcomes
- Outcome measures completed
- MHA Forms (if applicable)
- MHT Forms (if applicable)
- OCP approval email if emergency treatment (if applicable)
- SAT Guardianship Order (if applicable)
- Interpreter booked for first anesthetic assessment (if applicable)

If the documentation isn't available the day before treatment, escalate to the treating team (Consultant Psychiatrist/Registrar/Registered Medical Officer (RMO)) to ensure the above forms have been updated and completed by the Treating Team.

It is the responsibility of the treating Psychiatrist to review the patient's medications and recommend what medications need to be adjusted or ceased before commencing ECT. Benzodiazepines are usually withheld/ceased before ECT, discuss with treating team Consultant Psychiatrist/RMO

Refer to Appendix 1: Medications during ETC.

4.2 Night Staff Preparation of Patient

- All ECT documentation should be kept together and filed behind the ECT tab in the Medical Record.
- Night Shift Coordinator will check all ward diaries and send out appointment list to the CNS and Shift Coordinator.
- Ensure the patient fasts from midnight.
- Wake patient one hour prior to leaving for treatment and give any prescribed pre-ECT medications with sip of water.
- Complete 'ECT TREATMENT NURSING CHECKLIST' (PMR 74).
- Document baseline observations on the current Observation and Response Chart (ORC) chart which remains in the patient's file.
- Current patient files inclusive of Medication Chart (file), and Diabetes Sheet must accompany patient for treatment.
- **Check:** Consent Form; ECT Treatment Form (PMR 73); ECG tracing and Observation and Response Chart (ORC) are available and in the patient's file.
- Patient's jewellery to be removed or taped.
- Patient's hair to be clean and dry.
- Dentures/contact lenses can be removed in Isdell DPU treatment room.
- No moisturisers, face creams or makeup.

4.3 Day of ECT Preparation

Patient to be prepared one hour prior to leaving for treatment, ready to be transported by allocated Nurse to SCGH Mental Health Unit 'UU Block' (Isdell DPU), via Medic One / St John Ambulance.

***Note:** Medications related to management of blood pressure, reflux, asthma or cardiac stability must be given at least an hour prior to each ECT treatment, unless otherwise documented by Treating Team.

Other medications not related to ECT but supporting the wellbeing of the patient may be given at least an hour prior to treatment. (These may include antipsychotics for anxiety, or analgesics, please discuss with the team prior).

Allocated nurse to handover patient to ECT staff following ISoBAR protocols and is to remain with patient for duration of their treatment. Ensure all files are available and accompanies patient throughout treatment.

4.4 Post ECT

4.4.1 Post ECT (Prior to leaving ECT Suite)

All patients will be recovered in SCGH Isdell DPU by recovery staff until discharge criteria is met.

- The Escort Nurse will receive patient files and a verbal handover from Recovery Staff following ISoBAR protocols.
- Recovery will be approximately 2 hours post treatment.
- Patient will be provided a light breakfast as tolerated.

- Once reviewed by medical staff and discharge criteria is met, the patient will then be transferred back to hospital/ward of origin.
- Escort Nurse will be given a copy of the most recent Integrated Progress Notes which documents ECT treatment and recovery.

4.5 Post ECT Nurse Role on return to Inpatient facility

- On return to Selby/Osborne the ECT nurse will inform Medical Records of return time as the patient has been discharged for ECT and needs to be readmitted.
- ECT escort to inform the Shift Coordinator of return time.
- ECT escort nurse to complete a set of vital signs.
- ECT escort nurse is to inform RMO of patient's return to facility.
- ECT escort nurse to offer patient food and drink.
- ECT escort nurse to make an entry in the progress notes (PMR 7).
- ECT escort nurse to book transport for next ECT appointment.
- ECT escort nurse to inform Nurse Manager of next ECT date and to book escort nurse.
- When the patient is fully alert and orientated, encourage routine activities including attendance at the ward Occupational Therapy program.
- Documentation to reflect rationale for ECT Therapy e.g. notable changes in MSE.
- Encourage patient to use and bring "*Your Guide and Diary for ECT at SCGH Isdell DPU*" where any questions/queries can be discussed (also has useful phone numbers).
- The guide is given to all patients before the commencement of treatment, if the patient is unable to write themselves, it can be completed with/by the allocated nurse.

Refer to Appendix 2:Post ECT Inpatient Facility Checklist

4.5.3 Post ECT patients should be advised not to:

- Make major business decisions or sign contracts for at least 24 hours.
- Drive a motor vehicle or use heavy machinery for at least 24 hours.
- Use public transport unescorted.
- If a patient is requesting leave they will be reviewed by a doctor post ECT who will advise as to whether the patient is fit to leave hospital or not. If leave is granted, a responsible adult must then accompany them. They should be under the care of such an adult for at least 24 hours post ECT with access to a phone.

Note: This information is not a substitute for other clinical information and instructions from the patient's primary treating Consultant Psychiatrist, or other medical practitioner.

On completion of treatment ensure allocated team has informed ECT Coordinator / Team so bed can be reassigned.

4.6 Management of Ongoing ECT Treatment/Ceasing Treatment

4.6.1 Ongoing ECT Management

- The ECT nurse escort on their return to the ward will enter the patients next ECT appointment date and time into:
 - Shift Coordinator Diary
 - Wing Diary

- Medical Records Diary

- This will be confirmed with the Clinical Nurse Specialist.

4.6.2 Suspension, postponement or ceasing of ECT

- Ongoing/future ECT plans should be discussed regularly at weekly MDT meetings, with the patient (and family as appropriate) and with the ECT team. The ECT treatment plan should be clearly documented by the Treating team in medical records including the date for next ECT administration.
- When treatment is ceased, the treating Consultant Psychiatrist or medical delegate places a line through the ECT Consent Form (PMR 73) with the word “*ceased*”, dates and signs the entry.
- The treating Consultant Psychiatrist or medical delegate clearly documents this in the integrated progress notes that a decision has been made that ECT is postponed / suspended
- The treating Consultant Psychiatrist or medical delegate will provide a clear handover of the decision to postpone / suspend the ECT treatment to the CNS / Shift Coordinator/Senior Nurse on duty in a timely manner and prior to the next ECT.
- All future ECT appointments in the Shift Coordinator, Wing and Medical Records will be crossed out/cancelled by the CNS / Shift Coordinator / Senior Nurse on duty .
- The CNS / Shift Coordinator / Senior Nurse on duty will immediately add the postponement/suspension to the iSoft handover, cancel all future bookings in the central wing diary and all future bookings at the ECT suite.

5. Compliance, monitoring and evaluation

Compliance, monitoring and evaluation of this procedure will be conducted a minimum of annually through the committee of the Admitted Advisory Meeting.

Policy Sponsor	Co-Director Mental Health Specialities				
Policy Contact	MH Older Adult Specialities SRN7				
Date First Issued:	22/08/2023	Last Reviewed:	22/08/2023	Review Date:	22/08/2026
Approved by:	Co-Director Mental Health Specialities			Date:	24/07/2023
Endorsed by:	MHPHDS Executive Director			Date:	22/08/2023
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
National Standards for Mental Health Services	☐ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management ☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry				
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Physical Health Care of Mental Health Consumers, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
22/08/20223	V1.0	MH Older Adult Specialities, SRN 7, SRN 4 and SRN 3	Initial endorsement

This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2021

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism, or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.



6. Appendix 1: Medication during ECT

Medications during ECT

- It is the responsibility of the treating Psychiatrist to review the patient's medications and recommend what medications need to be adjusted or ceased before commencing ECT.
- Identify, in the patient integrated notes, which medications should be withheld during each ECT treatment, which medications should be given on the morning of ECT, and which medications should be continued post ECT. Amendments must also be made to the patient's medication chart. See Appendix xxx for recommendations for groups of medications.

Appendix: Specific information on management of common classes of medications during ECT

(adapted from: Oon H, Schwartz D. Medications and electroconvulsive therapy. Graylands Hospital Drug Bulletin. 2018 June; 25(1): 1-9)

These recommendations are to be used as a guide only, clinical judgement and reasoning will supersede the recommendations in this procedure.

Medication/Class	Risk with ECT Treatment	Recommended Action
Anticonvulsants	Potentially reduce efficacy by increasing seizure threshold	If used as mood stabiliser – cease medication if possible. Withhold dose from the night prior to ECT and can be administered after ECT treatment as soon as practical. Lamotrigine may be considered an exception and continued, due to its lesser effect on seizure induction and duration, difficulty in titrating dose, and risks associated with fluctuating plasma levels. If seizures are impacted, consider reducing the dose, or altering the timing of the dose to post-ECT (if being used once daily). If used for epilepsy – continue medication and withhold dose from the night before ECT
Benzodiazepines	Potentially reduce efficacy by shortening seizure duration	Cease if possible. If benzodiazepines cannot be discontinued, convert benzodiazepine with shorter half-life such as lorazepam and withhold from 2pm the day prior to ECT. Use non-benzodiazepines such as quetiapine for sleep/anxiolytic. Zolpidem/zopiclone can be used for insomnia due to their short half-life. Consider flumazenil pre-treatment prior to ECT if benzodiazepine cannot be discontinued or converted to alternative

		benzodiazepines with a shorter half-life. If benzodiazepine withdrawal symptoms occur within an hour post ECT, intravenous midazolam can be administered following ECT treatments
Antipsychotics	Antipsychotics (particularly clozapine and phenothiazines) lower the seizure threshold and would be expected to lead to seizures at lower ECT doses.	Combination of antipsychotics and ECT appears to be generally well tolerated and effective. Continue treatment with antipsychotic unless issues are encountered. Note: Although generally considered safe, Clozapine may carry a risk of prolonged or unwanted seizures, protracted delirium or serious hypersalivation. If issues are encountered, consider reducing the dose, or withholding for 24hrs pre-ECT.
Antidepressants	Potential of increase in seizure duration. Risk to clinical impact is minimal. Potential risk of asystole and cognitive adverse effects with venlafaxine doses $\geq 300\text{mg}$ Potential increase risk of cardiac arrhythmias with tricyclic antidepressants (TCA), especially in the elderly population and individuals with cardiac disease.	Continue treatment for all antidepressants unless issues are encountered except for: • Venlafaxine: Dose reduction to $\leq 225\text{mg}$ • Desvenlafaxine: Dose reduction to $\leq 150\text{mg}$ Consider risk of benefit of continuing TCAs, given that there are evidence of safety and improvement of cognitive outcomes with nortriptyline.
Lithium	Risk of cognitive disturbances, prolonged apnoea and spontaneous seizures. Recent evidence demonstrated safety with concurrent administration Theoretical interaction with suxamethonium prolonging its action.	Reduce lithium level to the lower end of the therapeutic range prior to ECT and withhold dose on night prior to ECT
Antihypertensives	Risk of hypertension during clonic phase of ECT if regular dose not administered prior	Administer dose with small sips of water prior to ECT with the exception of diuretics
Antianginals (beta-blockers, nitrates)	Risk of arrhythmias	Administer dose with small sips of water prior to ECT
Anticoagulants Antiplatelets	Possible risk of intracerebral haemorrhage	Generally safe to continue anticoagulant and antiplatelet agents during a course of ECT as the risk of stopping these medications and running into

		cardiac issues would appear to vastly outweigh the tiny risk of intracerebral bleeding
Diuretics	Risk of incontinence during seizure	Withhold dose on morning of ECT and administer post ECT
Asthma medication (preventers)	Risk of asthma exacerbation during ECT if not administered	Administer dose prior to ECT with the exception of theophylline
Theophylline	Risk of prolonged seizure or status epilepticus	Cease prior to ECT if possible, or maintain at lowest therapeutic dose if ceasing medication not appropriate
Antidiabetic medications	Hypoglycaemia as patient fasting from night before ECT	Withhold all oral antidiabetic medications, short-acting insulin, intermediate-acting insulin and incretin mimetics (e.g. exenatide) on the morning of ECT and administer post ECT when patient can start eating. If basal insulin (e.g. glargine) is administered: In the evening - Continue usual dose night prior to ECT. In the morning - Administer 50% of the usual dose the morning of the procedure and administer 50% of the usual dose once the patient starts eating post ECT.
Gastrointestinal Medications	Risk of reflux and potential aspiration during ECT if not administered	Administer dose with small sips of water prior to ECT

7. Appendix 2: Post ECT Inpatient Facility Checklist

Post ECT Inpatient facility checklist

TO BE COMPLETED BY ESCORT NURSE FOR ECT UPON RETURN TO SELBY

Inform medical records of patient return 767	☐
Inform the Shift Coordinator on 706	☐
Offer patient food and drinks	☐
Complete a set of Patient vital signs	☐
Give patient their regular medication	☐
Call the RMO on 749 to inform them of patient return	☐
Please book Transport on 130026200 Medic for the next ECT	☐
Inform nurse manager of next ECT date and book escort nurse	☐
Document in the patient progress notes	☐